



Tort Law (9th edn)

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<https://doi.org/10.1093/he/9780198925231.003.0012>

Published in print: 15 August 2025

Published online: August 2025

Abstract

This chapter discusses the ways in which liability for defective products is regulated in tort law and which is primarily regulated by statute. The chapter considers manufacturers' liability in negligence and the problems claimants may encounter when trying to sue. This leads to a later development relating to product liability—the Consumer Protection Act (CPA) 1987—enacted to ensure better protection of consumers from defective goods by making producers strictly liable for harms caused by the products they market. However, a claimant may still claim in negligence due to the statutory limitations in relation to certain types of claim.

Keywords: Keywords, tort law, defective product, manufacturers' liability, negligence, Consumer Protection Act, defect, state of the art defence

Problem question

Read this problem question carefully and keep it in mind while you are working through the chapter that follows. At the end of the chapter you will be able to apply what you have learnt to the problem question and advise the relevant parties.

After many years of research, Rack and Horse Pharmaceuticals (RHP) developed a drug to treat breast cancer. After only 18 months of clinical trials, it received a licence and went on the market in the UK in March 2020. Although the drug itself is completely pure, it is now known that in less than 0.5 per cent of patients (those who carry a particular gene) it can produce an undesirable side effect known as Tort Syndrome. This side effect is not widely publicised as both RHP and the government are keen to encourage widespread uptake of the drug in relevant groups of women.

In 2025, 20 claimants who were given the drug between 2020 and 2022 and who contracted Tort Syndrome begin an action against RHP alleging both negligence and liability under the Consumer Protection Act 1987. RHP argues against liability because up until 2023, there was no genetic test that could determine which individuals carried the gene in question.

The claimants bring evidence to show there was an article in an Outer Mongolian scientific journal, published both in hard copy and on the internet in 2021, which suggested a test to determine whether individual women carried the specific gene for the reaction to the drug that causes Tort Syndrome. Had RHP conducted clinical trials for longer, the company would have been able to identify the characteristics of the women likely to react badly to the drug and to issue appropriate warnings and advice.

Advise the parties.

p. 350 12.1 Introduction

Consider the following examples:

→ *Stuart buys Bluetooth headphones from an electrical store. He later finds out that they do not work because a small but crucial component is missing.*

→ *Kristian buys Bluetooth headphones from an electrical store. He later tries to charge them but the internal wiring is faulty; he receives a small electric shock.*

→ *Theresa receives Bluetooth headphones for her birthday. She later finds out that they do not work because a small but crucial component is missing.*

→ *Amarpreet buys a car for Marianne. Two weeks later, one of the tyres explodes while Marianne is driving, causing the car to swerve into her garden wall, which will cost £500 to rebuild. The car costs £1,000 to repair.*

→ *Rosie buys a car for Molly. After a year, the two front tyres need replacing. Two weeks later, one of the replaced tyres explodes while Molly is driving, causing the car to swerve into her garden wall, which will cost £500 to rebuild. The car costs £1,000 to repair.*

→ *Tyler takes a drug to stop his head aching, but later finds out that he has a stomach ulcer caused by an unusual reaction to the drug.*

At first sight, it might seem odd that we are considering product liability in a book on tort law, when most of us are more accustomed on a day-to-day basis to dealing with ‘products’ and the consequences of their defects via contract law. However, *Donoghue v Stevenson* [1932], the foundational case for all modern claims in negligence, is itself a case about a defective product (contaminated ginger beer) that harmed the end-user (gastroenteritis).

Liability for defective products is, therefore, also possible in negligence, though the picture is, as we shall see, more complicated than might be expected. Much depends on what type of defect is being claimed for. It is more appropriate to sue in contract for some harms caused by defects in products and in negligence for others. In basic terms, this depends on whether the claimant takes their case against the retailer (contract) or the manufacturer (negligence) of the item. Often, a claim in contract might be preferable where possible, as ‘strict liability’ (i.e. liability without the need to establish fault) is imposed by statute on retailers in respect of the quality of the goods that they sell, and the remedies are set out. When claiming in negligence against a manufacturer, the usual hurdles of duty, breach and—in particular—causation must be overcome.

Because an understanding of liability for defective products in contract law is an important part of the overall picture regarding defective products in law, it is worth either refreshing your memory about these liabilities, or gaining a brief overview of them, if you have yet to study contract law, before reading on. To help you, we have included a basic summary of the liabilities that may arise in contract on our [online resources](#).

This chapter looks at *tortious* liabilities for defective products, beginning with manufacturers’ liability in negligence and the problems claimants may encounter. This leads us to a later development relating to product liability—the Consumer Protection Act (CPA) 1987—enacted to ensure better protection of consumers from defective goods by making *producers* strictly liable for harms caused by the products they market.

p. 351 12.2 Defective products—Claims in negligence

As already mentioned, at its narrowest interpretation, *Donoghue* is a landmark product liability case. The case not only lays the foundations of the entire tort of negligence but also specifically allows for claims in negligence to be taken directly against the manufacturers of defective products.¹

Before *Donoghue*, a person injured (either personally or through damage to their property) by defective goods had limited grounds for recovery. If they had a contract with the retailer of the goods then they may have had a claim for breach of contract and (subject to the various rules governing this type of claim) compensation could be awarded for both the cost of replacing the product itself *and* of putting right any damage to person or property caused by the product. Contract law, however, is limited in the protection it can offer—not least only (usually) to those who are party to the contract, which Mrs Donoghue was not. Further, in the tort of negligence, though someone in Mrs Donoghue’s position could be compensated for their illness and distress, the cost of the *product itself* is regarded as pure economic loss and, as such, unrecoverable.² The claimant could only have received compensation (in contract) covering the cost of the ginger beer itself if she had paid for it. If, as was the case in *Donoghue*, she had no contract with the seller, she would be limited to a claim in negligence against the manufacturer.

Before 1932, manufacturers could only be liable in relation to products if the product was classed as ‘dangerous’ or was manufactured to be dangerous, in which case a warning had to be given. However, the distinction between ‘dangerous’ and ‘not-dangerous’ was not always helpful.³ In *Donoghue*, the House of Lords overturned the distinction and also discarded what it termed the ‘privity of contract fallacy’, holding that there should be no reason why the same facts should not give one person a right in contract and another person a simultaneous right to sue in negligence. It drew no distinction between the type or nature of the product (though it seems logical that this would be a relevant factor when assessing the *standard* of care to which the manufacturer should be held and whether that has been breached). That is, it seems common sense to say that manufacturers of inherently dangerous products ought to be even more careful.⁴

Pause for reflection

The law lords in *Donoghue* were split 3:2 in favour of the claimant. Two dissented, mainly because of concerns about opening the ‘floodgates’ to claims (similar reasoning is often seen in modern negligence cases, where a court is asked to expand the types of situation or relationship in which a negligence claim might succeed). Within the majority opinions, Lord Macmillan took a more pragmatic approach to the finding of liability than Lord Atkin’s broad ‘neighbour’ principle—favouring a more cautious case-by-case approach (similar to that used in novel claims following *Caparo Industries plc v Dickman* [1990]).⁵

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↪ Interestingly, three years prior to *Donoghue* a case with remarkably similar facts had been dismissed. In *Mullen v AG Barr* [1929], it was alleged that a bottle of ginger beer contained a decomposing mouse but, as there was no precedent for the type of claim being made (ginger beer not being ‘dangerous’ in and of itself), the case failed. The same lawyers represented Mrs Donoghue—and were clearly very persistent—so it is them we should thank for the law as it stands today!

Generally, the wide scope of the tort of negligence (and negligence in relation to product liability) is these days taken for granted and the impact and importance of the *Donoghue* decision is not really seen amongst all the later developments, limitations and rules. Consider how negligence might look if the claimant in *Donoghue* had failed or if Lord Macmillan’s approach had been favoured over Lord Atkin’s.

12.2.1 The scope of liability under *Donoghue*

As *Donoghue* is a negligence case, liability in relation to harm caused by products must be assessed by considering the various factors—duty, breach and causation—that a claimant must establish when making any claim in the tort of negligence. As these have all been discussed in far greater detail in **Chapters 2 to 10**, we concentrate here only on the more pertinent issues regarding product liability.

12.2.1.1 Duty of care

The duty owed is the ordinary common law duty of care—based on the presumed intention to supply the goods to the ultimate consumer in the same state they left the production line. The duty extends beyond the manufacturer/consumer relationship to include, for example, packers, machine operators and distributors. Similarly, the ‘ultimate consumer’ not only covers the ‘end-purchaser’ of the product concerned but includes also the ultimate user (as in *Donoghue*) and any person coming into contact with the product. In *Stennett v Hancock & Peters* [1939], for example, the manufacturer’s duty of care extended to a pedestrian who was hit by a wheel of a lorry when it fell off.

The duty may also extend *beyond* the product itself to include, for example, containers, packaging or instructions (*Watson v Buckley, Osborne, Garrett & Co Ltd* [1940]). In *Donoghue*, Lord Atkin defined the type of products that the duty covers as ‘articles of common household use, where everyone, including the manufacturer, knows the articles will be used by persons other than the ultimate purchaser’ (at 46). Clearly, however, given the example of *Stennett*, mentioned earlier, the definition of ‘products’ must extend beyond ‘household items’ and could, logically, include *any* product where the definition of ‘consumer’ is taken in its broadest sense.

Despite the range of relationships covered and a broad understanding of what a product is, it should be noted that the principles *preventing* a duty of care from arising in the wider law of negligence also apply here.

Damage to (or a defect in) the product itself, for which the only claim is the cost of replacement, is regarded as pure economic loss, for which no duty can arise (*Murphy v Brentwood District Council* [1991]).⁶ A claimant can therefore recover damages for anything the defective product causes injury to, but not for any injury to the product itself. The remedy in tort would seem, in this respect, to be less useful than that in contract.

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Pause for reflection

Is there a good reason for regarding the cost of replacing the item itself as pure economic loss, thereby preventing a remedy in tort? Does it seem fair that if you bought the item for yourself, you would be able to claim this cost against the retailer (in contract), but if you were given the product as a gift, you could not obtain a replacement? While it might be thought that the person who bought the present could always bring a claim on your behalf (e.g. the position of Amarpreet in the examples at the beginning of this chapter), in reality they cannot as they will have suffered no harm.

Before we leave ‘duty’ some mention should be made of what is known as the ‘complex structure’ theory.⁷ Under this theory, component parts of a product are (sometimes) seen as a separate product entirely.⁸ Therefore, if the component part is the defective ‘product’, and it causes damage to the rest of the product or another part of it, this can be classed as consequential damage and not pure economic loss (though the cost of the defective component itself would be pure economic loss).⁹ Consider how this relates to the Amarpreet/Marianne and Rosie/Molly situations outlined at the beginning of the chapter. If the tyre exploded soon after

the car was purchased, then it is likely that the courts would view this as a defective car and Marianne would have no claim (she did not buy the car herself so cannot claim against the retailer) for the cost of fixing the car, although she would be able to claim for the cost of rebuilding her wall. However, if Molly's replacement tyre exploded, this could be considered to be one component of a 'complex structure' (the car) and subsequently any damage to the car, as well as the wall, would be consequential loss.¹⁰ All that Molly would be unable to claim would be the cost of replacing the tyre.

Pause for reflection

Should a manufacturer's duty extend to taking steps to recall defective products that have already gone into circulation?

In the US case *Grimshaw v Ford Motor Co* [1981], Ford was alleged to have discovered a defect in the Ford Pinto which sometimes caused the cars to explode if they were hit by another car from behind. However, the company calculated that it would be cheaper to pay compensation to anyone injured or killed in such an explosion than it would be to recall all the cars in circulation and repair the defects. If there had been a duty to recall the cars once the defect was known then Ford would have been in breach for not doing so. At trial, Ford was ordered to pay punitive damages in excess of \$125 million, though this was reduced to \$3.5 million on appeal (plus compensatory damages of just under \$3 million).¹¹ This case illustrates many of the problems associated with using private law remedies in relation to product liability. The fact that producers can be sued is meant to have a deterrent effect, thus changing their behaviour. Yet, it seems, sometimes producers (particularly large-scale producers) will weigh the risks and benefits to their profits and shareholders when considering whether to recall and rectify defects in their products (even where these are dangerous enough to pose a risk to life, as in the *Ford* case). In this sense, it seems that stronger public law regulation might be a better deterrent.¹²

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12.2.1.2 Breach and causation

Breach of duty of care was considered in **Chapter 8**. The question here is: did the defendant manufacturer exercise reasonable care when making the product? This is, as ever, to be assessed on a case-by-case basis.

Breach is not normally problematic as it may often be inferred from the presence of the defect. That is, it is often assumed that any defect can only exist because there was negligence during the manufacturing process—how else could a snail come to be in a sealed bottle of ginger beer, for example? The manufacturer must rebut the presumption of carelessness with evidence. This principle can clearly be seen in *Grant v Australian Knitting Mills Ltd* [1936].

***Grant v Australian Knitting Mills Ltd* [1936] AC 85 (PC)**

Mr Grant purchased some woollen underpants and, after wearing them, suffered from a painful skin condition. This was caused by a reaction to sulphites present in the wool, left over from the manufacturing process. The claimant could not prove negligence on the part of the manufacturer and the manufacturer claimed that there had been no carelessness during the manufacturing process, and that reasonable care had been taken to ensure that chemicals used did not remain in the wool.

The court found that there could be no other explanation for the sulphites in the wool than that one of the employees had, at some point during the manufacture of the underpants, been careless. The manufacturers were therefore held liable.

p. 355 Defects in products that are present when the product reaches the end-user can be inferred to have been caused by the manufacturer's carelessness, unless the manufacturer did not intend the product to reach the end-user in the same condition it left the factory. Following *Grant*, it is relatively easy to establish a breach of duty, as courts are willing to infer carelessness without evidence to the contrary. This inference is not always made, however. In *Evans v Triplex Glass Co Ltd* [1936], for example, a claim against the manufacturer of windscreen ↵ safety glass did not succeed as the court found that the fault was more likely to lie with the fitters of the glass than its manufacturer.

Once breach is established, it must be decided whether the breach actually caused the harm complained of. The tests for causation in fact and in law are the same in product liability as for any other area of negligence.¹³ In a sense, however, the duty requirement that the item must not have been subject to intermediate examination between leaving the manufacturer and reaching the end-user can also be viewed as a breach/causation question. To find the defendant liable, a court must satisfy itself that they were responsible for the defect and that it was not due to the fault of someone else further down the chain of supply. If someone else—for example, a distribution company who inspected the goods before they were supplied to retailers—could be said to be responsible for the defect, this will 'break the chain' of causation back to the manufacturer. In addition, an end-user of a product will be unsuccessful in a claim if it can be shown that they had knowledge (actual or constructive) of the defects.¹⁴

In *Grant*, the defendants argued that the goods may have been tampered with after leaving the factory and the fact that this might have happened (there was no real way of proving whether it had) should enable the manufacturer to escape liability. They argued that because the garments had been wrapped in paper packets and could be sold separately, there was a possibility that they had been interfered with. The court dismissed this argument, finding that interference was a question of fact (i.e. evidence must be shown to prove that there was actually interference, otherwise the presumption will be that there was none). However, *Evans* (decided in the same year) tells us that if there is a 'reasonable possibility' or probability of interference with the product, this will be enough to exonerate the manufacturer from liability.

The question of inspection may be a difficult one, especially if a party in the supply chain had the *opportunity* to inspect the goods but did not take it. There is no obligation on intermediate parties (or, indeed, the end consumer) to exhaustively examine goods that pass through their control, so a manufacturer will remain liable unless they have a genuine or specific reason to believe that the goods will be inspected by another party before they leave the supply chain. This, of course, prompts another question: when *can* a manufacturer reasonably expect another party to examine the goods? While in *Andrews v Hopkinson* [1957] a second-hand car salesman had the opportunity to check cars and was held liable for failing to do so for any obvious faults (although a full and exhaustive inspection was not required), in *Hurley v Dyke* [1979] a seller of a defective second-hand car avoided liability as he had informed the buyer that it was ‘as seen and with all its faults’ (the warning moved the duty onto the buyer to inspect the car for himself).

12.2.2 The limits of negligence liability

So far we have considered the position under *Donoghue* in relation to manufacturing defects. However, products can be also defective because of the way they are *designed* and in these circumstances we begin to see the limits of a claim in negligence. This is problematic. Design defects by their very nature are likely to affect greater numbers of end-users—possibly even every consumer of a product—and are also likely to be difficult to discover by examination of the product either before it leaves the supply chain or by the ultimate consumer. In short, design defects are more serious and increase the possibility of harm to the product’s end-users.

p. 356 ↩ Unfortunately, it is also more difficult for someone to succeed in a negligence claim in relation to design defects. While duty is easy to establish, it is difficult to show that there was carelessness in designing a product, that is, that the designers fell below—or breached—the standard of care expected of them. In relation to causation, the claimant must show two things: first, that the design defect *can* cause harm and, secondly, that the design defect *was* the actual or material cause of the specific harm being claimed for. This is a high threshold. For example, a claimant may be able to show that a particular drug, because of a design flaw, can cause cancer. However, it will be very difficult for them to prove, on the balance of probabilities, that taking the drug caused *their particular cancer*, particularly as other factors such as environment, genetics and lifestyle may also all be relevant (as in *Wilsher v Essex Area Health Authority* [1988]).¹⁵

The particular difficulties associated with design defects became very apparent in the 1960s with the Thalidomide tragedy.

The Thalidomide tragedy

Thalidomide is a drug that was initially sold from the mid-1950s to 1961 in many countries worldwide and under many different product names. It was primarily prescribed to pregnant women, to help combat morning sickness and to help them sleep. However, it later transpired that Thalidomide is a potent teratogen, which means that it can cause severe birth defects if taken during pregnancy. The

drug had been inadequately tested before being put on the market and approximately 10,000 children were born with birth defects after their mothers took it while pregnant. These defects included phocomelia, characterised by the shortness of the long bones in the arms or legs and therefore often severely shortened limbs, as well as problems with the heart or kidneys, or with digestion. Life expectancy was also shortened and many affected children died in infancy.

In 1962, in reaction to the tragedy, the US Congress enacted laws requiring tests for safety during pregnancy before any drug can receive approval for sale in the United States and numerous other countries enacted similar legislation. Thalidomide was not prescribed or sold for decades.¹⁶ However, perhaps surprisingly, it was never withdrawn completely from the medical marketplace and is used today to treat a number of conditions, including leprosy and several cancers.

The Thalidomide tragedy is a perfect example of harm caused by a design defect. Nothing in the *manufacturing* process had gone wrong, causing a particular batch of the drug to produce negative side effects in those for whom it had been prescribed. Instead, part of the *design* process had been faulty. The parents of some of the children affected by Thalidomide claimed in negligence against the producers of the drug. The legal outcomes of their claims are forever unknown as they were settled before reaching court, however it is clear that although the manufacturers owed those affected a duty of care as the ultimate consumers, because the consequences of the drug were not foreseeable by a reasonable manufacturer at the time it was circulated, it would have been very difficult for them to show that the manufacturer had breached this duty by failing to take reasonable care in the design process. It would have also been difficult to establish causation—the birth defects could have been caused by a number of different things.¹⁷

What the Thalidomide tragedy did highlight was the need for change. While the common law was in a fairly satisfactory position in relation to manufacturing defects, as courts were willing to infer negligence (*Grant*) and generally had a fairly ‘flexible’ approach to liability, the Thalidomide cases showed what problems claimants might face if trying to claim for harms caused by *design* defects. This led to demands for legal reform and, in particular (perhaps because it was ‘fashionable’ at the time), a call for ‘strict liability’ to be placed on manufacturers of defective products.

Strict liability is liability without fault. What it would mean for manufacturers to be strictly liable for defective products is that even if they did nothing wrong in the manufacturing or design process (i.e. there was nothing that could technically be called a ‘breach’), they would have to compensate someone who suffered harm caused by a defect in their product. The main reasons suggested for introducing strict liability were threefold:

- (1) manufacturers create products and therefore create the hazard or harm. Since this is done in the pursuit of profit, it is reasonable for manufacturers to accept the risk of liability for any harms caused by the product;
- (2)

manufacturers are in the best position to insure against the risks of any such hazards or harms (and the price of such insurance can be reflected in the product price and therefore distributed amongst all end-users of a product);

- (3) imposing strict liability on manufacturers provides an incentive for them to take all possible safety precautions during the design and manufacturing processes.

In light of increasing pressure worldwide in favour of strict liability, the European Community issued a Directive in 1985¹⁸ requiring member states to change and harmonise their laws on product liability.¹⁹ In the UK, this took the form of Part 1 of the CPA.²⁰

Test your understanding by trying this section's self-test questions <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-12-2>.

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12.3 Defective products—Claims under Part 1 of the Consumer Protection Act 1987

Section 1(1) of the CPA stipulates that:

This Part [was enacted] for the purpose of making such provision as [was] necessary in order to comply with the product liability Directive and shall be construed accordingly.

Thus, it may be that it is the terms of the Directive itself that should be followed, rather than the terms of the Act, as the CPA must be construed in line with the original meaning intended by the Directive—a point to which we shall later return.

The CPA makes the manufacturer of a product (and others along the supply chain) liable *without proof of fault* (strictly liable) for personal injury and some property damage caused wholly or in part by a defect in the product concerned. This looks to be a step forward from, and a contrast to, the tort of negligence, where fault must be proved. As formulated, strict liability as (supposedly) provided by the CPA ought to provide an easier route to redress.

Manufacturers are, however, granted a number of specific defences under the CPA. These are problematic. The imposition of strict liability is considerably less strict if the defendant is able to defend claims brought against them. One of the defences in particular has caused considerable controversy, as we shall see later.

Essentially, establishing liability under the CPA requires a series of questions to be asked. The Act defines concepts such as 'product' and 'defect' and it is within these confines that claims must proceed.

12.3.1 Bringing a claim

Unfortunately, the CPA does not clearly set out who can sue and who can be sued.²¹ Reading between the lines, it appears that anyone who suffers damage covered by the Act as a result of a defective product can claim (ss 2(1) and 5(1)). The definitions in section 1(2) and a further outlining of who potential defendants might be in section 2(2)(a)–(c) suggest that manufacturers and producers of goods can be sued, as well as ‘own-branders’ (companies who put their name to a product made for them by someone else)²² and importers. Similarly, *suppliers* of goods can be sued if the conditions in section 2(3) are met. The purpose of the Act is to increase the protection of consumers (in the spirit intended by those advocating strict liability for defective products) by ensuring that *someone* can be found liable in relation to defective products. This is underlined by section 2(5) which states that if more than one potential defendant can be liable for the same damage they will be jointly and severally liable.

To claim, the right ‘kind’ of damage is needed. This is defined in section 5 as death or personal injury, or property damage with a value of more than £275 (s 5(4)). Thus, small monetary claims for property damage are excluded (perhaps to limit the number and ensure the ‘seriousness’ of claims). There is no upper limit to the total amount of damages that may be claimed (despite an option to limit this when implementing the Directive). Pure economic loss is also specifically excluded by section 5(2), which means that, as in the tort of negligence, the cost of *replacing* a defective product is not recoverable.

Pause for reflection

Does a figure of £275 as the minimum claimable limit for property damage seem (a) quite arbitrary and (b) relatively low? Consider what you can buy for £275 these days and what could have been bought for that sum when the CPA came into force. Should this figure be increased?

‘Products’ are defined in section 1(2) and include ‘any goods or electricity and ... a product which is comprised in another product, whether by virtue of being a component part or raw material or otherwise’. ‘Goods’ are further defined (s 45) to include ‘substances, growing crops and things comprised in land by virtue of being attached to it, and any ship, aircraft or vehicle’. This definition is quite broad and possibly encompasses things that, as a consumer, one might not ordinarily have considered.²³

The key aspect of the CPA is its coverage of ‘defective’ products—so the definition of defect is one that needs detailed analysis. It is here that most of the case law under the CPA can be found: the courts have found themselves tasked, on occasion, with ruling on whether or not a certain product is ‘defective’ for the purposes of the Act.

12.3.2 What is a 'defect'?

Defects in products for the purposes of the CPA are defined in section 3. As a defendant is strictly liable for any damage (of the right kind) caused wholly or in part by a 'defective product', this definition is a key part of the Act. Section 3 defines a defect as follows:

[T]here is a defect in a product for the purposes of this Part if the safety of the product is not such as persons generally are entitled to expect ...

The burden of proof is on the claimant to show that a defect exists, and that this caused them harm. In addition, defects are about safety, not quality. However, it seems that the standard (regarding what is and what is not defective) is set by 'persons generally'—not, for example, by manufacturers, consumers or any other specific group or individual (see *Richardson v LRC Products Ltd* [2000]). But the way the definition is phrased does mean that, objectively, the perspectives of those groups or individuals have to be taken into account when considering what 'persons generally' might think. This is a rather weak and circular definition. Indeed, in *Wilkes v DePuy International Ltd* [2016], Hickinbottom J pointed out that as an objective, rather than a subjective, *consumer* expectation test, it is rather an 'empty vessel' (at [68]).

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Counterpoint

If the CPA really is about consumer protection, it seems odd that the expectations of manufacturers and others with vested interests should be taken into account when deciding whether a product is 'defective'. Manufacturers base their considerations on a cost/benefit analysis—for example, to them, a product will be 'defective' only if the magnitude of the danger it poses and the cost of preventing any harm occurring outweigh the product's utility—this risk/utility analysis is more familiar to us from the tort of negligence and in particular an assessment of the standard of care and whether a defendant has fallen below it. This, therefore, implies *fault*—and the CPA was meant to bring no-fault (strict) liability.

On the other hand, it is questionable whether strict liability is desirable in this area at all. Manufacturers have to absorb the costs and risks of development in other ways and, unless clearly negligent in the way they make a path to market (e.g. if a pharmaceutical company did not undertake any testing to determine whether a drug it was producing had any harmful side effects), it can seem unreasonable that they also have to pay for harms suffered by people using their products. Consumers generally welcome choice, innovation and competition (more products on the market, prices are kept down by competition, etc), so is imposing strict liability on producers allowing consumers the best of both worlds? Perhaps it is arguable that consumers should take more responsibility for themselves when it comes to injuries suffered in this context (see *Abouzaid v Mothercare (UK) Ltd* [2000] and *Pollard v Tesco Stores Ltd* [2006] as just two examples of cases where it might well be argued that had more care been taken by the consumer, the risk would not have materialised).²⁴

Evidently, the CPA's definition of 'defect' allows products into the market that are potentially dangerous—but not defective. As the concept of a defect is linked to 'safety' and about what may be reasonably expected, this means that certain items that are inherently dangerous cannot be classed as defective. For example, a cut-throat razor or a serrated bread knife will not be 'defective' products, as people are 'generally entitled to expect' that razors and knives will be sharp.

Pause for reflection

Where does this leave products such as cigarettes and alcohol? Are these as safe as 'persons generally are entitled to expect'? If so, what makes them this 'safe'? It could be argued that the damaging effects of cigarettes and alcohol are well publicised. For example, the potential consequences of smoking are already clearly pointed out on packaging,²⁵ and government ↵ campaigns have highlighted that people who consume alcohol should take into account how many 'units' they consume, with products now labelled to allow consumers to find this information easily. It is very unlikely that policy-makers would think it wise or practical to take tobacco and alcohol out of circulation (consider the 'Prohibition' experiment in the United States during the 1920s and 30s, which drove the supply of alcohol underground and created a criminal subculture). There are cultural issues with regulating an existing product which has been found to be dangerous long after release, as opposed to regulating the safety of a new product coming onto the market. So, while it might be said that tobacco and alcohol are not as 'safe' as other products, public expectations in relation to these substances clearly differ greatly from those in relation to others.

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The question when deciding whether a product is defective for the purposes of the CPA is: what *are* 'persons generally entitled to expect'? As this is an objective standard it can be difficult to assess, although section 3(2) (a)–(c) sets out guidelines as to what factors may be taken into account when considering the safety/defectiveness of a particular product, including:

- (1) the manner in which, and purposes for which, the product has been marketed;
- (2) the 'get-up' (packaging);
- (3) use of marks (e.g. the British Standards Institution 'Kitemark');
- (4) instructions or warnings accompanying the product when sold;
- (5) what might reasonably be expected to be done with the product; and
- (6) the time at which the product was supplied.

Counterpoint

The issue of defectiveness or non-defectiveness is perhaps a common-sense interpretation of the safety of a product, according to how and why it is marketed, how it is packaged, what people (consumers) are meant to do with it (and what they are not) and, importantly, any warnings accompanying the product. It can be assumed that an adequate warning would be enough to make an otherwise defective (i.e. unsafe) product non-defective. This, too, does not sound like strict liability in any real sense. Either a product is unsafe or it is not—should giving a warning be enough to remove liability? Put another way, if there is a product which would be deemed unsafe if no warning came with it, are we saying the manufacturer is *at fault* for not supplying an adequate warning, and should therefore be liable? Conversely, if a warning is included, is there no fault and therefore no liability? Is this really a form of negligence?

p. 362 When considering ‘defectiveness’, the question of whether adequate warnings were given (thus rendering a product ‘safe’) has arisen in case law generated by the CPA (of which there is surprisingly little). For example, in *Worsley v Tambrands Ltd* [2000], a woman complained that a tampon she had used was defective when she suffered toxic shock syndrome after using it. She claimed that while it was known that the risk of this syndrome appearing was greater with misuse of the product, she had used it according to the instructions. As the evidence showed that she had read the comprehensive instructions, including the warnings given about the product, the court held that the tampon had not been defective as ‘people generally’ (in this case, women using the product) should know, as it was stated in the instructions and warnings, that a small risk of toxic shock syndrome was present even with ordinary use. Similarly, in *Richardson v LRC Products Ltd* [2001], the question for the court was whether a condom, from which the whole tip had sheared off during use (i.e. it had more than just split or punctured during sex), was defective. The claimants were claiming for the costs associated with pregnancy and birth of an unwanted child. Again, because it was considered well known that condoms are not 100 per cent effective, due to the warnings to this effect that always accompany them (including on the outer packaging), the condom was not found to be defective.

Counterpoint

Richardson may be viewed outside the ambit of product liability as a case where a claim for ‘wrongful birth’ was being made. In other cases of this type (e.g. when a sterilisation operation has been performed negligently), the House of Lords was adamant in its view that this is not a compensable harm.²⁶ While the courts are prepared to award damages for the pain and suffering associated with the ‘physical harm’ of pregnancy and birth caused by negligence, damages for ‘wrongful birth’ are generally unavailable. This is largely due to policy reasons: either the harm suffered is not viewed as a legal harm, or the harm (the cost of raising an unwanted child) has been deliberately classified as ‘pure economic loss’ (see *McFarlane v Tayside Health Board* [1999]—discussed in **section 7.5.2**). Given

the date of *Richardson*, it may well be assumed that this decision also had the weight of the public policy arguments from the wrongful birth negligence cases behind it, even though the same harm was caused in a different way.

Read more about this counterpoint [_<https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-12-pointers-to-pause-for-reflection-and-counterpoint-boxes?options=showName>.](https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-12-pointers-to-pause-for-reflection-and-counterpoint-boxes?options=showName)

In *Bogle and others v McDonald's Restaurants Ltd* [2002], customers of McDonald's fast-food restaurants sued the company (in negligence as well as under the CPA) for scalding caused by hot coffee. The product (the coffee) was argued to be 'defective' in two ways: first, the temperature at which it was served was too high and, secondly, the lids on the cups in which it was served came off too easily. The High Court dismissed the claims, holding that 'persons generally' would know that hot coffee could cause scalding if spilt and that care should be taken with hot drinks to avoid spillages. Because people want hot coffee, they have to suffer the 'inevitable' risk that comes with it. Field J said that:

p. 363 if McDonald's were going to avoid the risk of injury by a deep thickness burn they would have had to have served tea and coffee at between 55C and 60C. But tea ought to be brewed with boiling water if it is to give its best flavour and coffee ought to be brewed at between 85C and 95C. Further, people generally like to allow a hot drink to cool to the temperature they prefer. Accordingly, I have no doubt that tea and coffee served at between 55C and 60C would not have been acceptable to McDonald's customers. Indeed, on the evidence, I find that the public want ↵ to be able to buy tea and coffee served hot, that is to say at a temperature of at least 65C, even though they know (as I think they must be taken to do ...) that there is a risk of a scalding injury if the drink is spilled. (at [33])

Put another way, the decision says that coffee-buying customers (who were usually adults and teenagers) should generally expect coffee to be hot and should guard against spillages themselves. On the second issue, the court found that the lids needed to come off easily to allow customers to put sugar and milk in their drinks; the fact they came off easily if the cups were knocked over did not make them defective. The test used here seems to be one of 'legitimate consumer expectation' and can probably be linked back to the use of the words 'entitled to expect' in section 3 of the CPA—that what persons generally are 'entitled' to expect may well be different from what an individual claimant or claimants did in fact expect (i.e. some people may have higher standards, but they may not be 'entitled' to these standards under the CPA).

Pause for reflection

Bogle may be compared to the infamous US case of *Liebeck v McDonald's Restaurants* [1994], which is often used as an example of the US 'compensation culture'. The case became a flashpoint in the US debate over tort reform after a jury awarded nearly \$3 million to a woman who was burned by hot coffee bought from McDonald's.²⁷ Stella Liebeck, a 79-year-old woman, bought a cup of coffee from the drive-through window of her local McDonald's restaurant. She was sitting in the passenger seat of her grandson's car, which he had parked to allow Mrs Liebeck to add cream and sugar to her coffee. She placed the coffee cup between her knees and pulled the lid towards her to remove it, spilling the entire cup of coffee on her lap and burning herself in the process. Her award was reduced to \$640,000 by the trial judge, though the case was later settled out of court for an unknown amount (thought to be less than \$600,000) before an appeal was decided.

Do you think *Bogle* is evidence of a 'compensation culture' in the UK? Or do you think the claims were legitimate? Some often unreported information (or facts that are conveniently forgotten by those arguing that a compensation culture exists) about the *Liebeck* case is that the coffee, which had been absorbed by Mrs Liebeck's clothing, was then held against her skin as she sat in the puddle of hot liquid for over 90 seconds, scalding her thighs, buttocks and groin. She was taken to hospital, where doctors determined that she had suffered third-degree burns on 6 per cent of her skin and lesser burns over 16 per cent. She remained in the hospital for eight days while she underwent skin grafting, which was then followed by two years of further treatment. During the trial, the court heard that McDonald's required its franchises to serve coffee at 180–190°F (82–88°C). At that temperature, it was said that the coffee could cause a third-degree burn in between two and seven seconds. Other evidence showed that from 1982 to 1992 the company had received more than 700 reports of people burned by its coffee, to varying degrees of severity, and had previously settled claims arising from scalding injuries for more than \$500,000.

Mrs Liebeck had not intended this outcome: initially, she sought to settle with McDonald's for \$20,000 to cover her medical costs, which were \$11,000, but the company offered only \$800. When McDonald's refused to raise its offer, she claimed against them, alleging 'gross negligence' for selling 'unreasonably dangerous' and 'defectively manufactured' coffee. McDonald's then refused her lawyer's offer to settle for \$90,000. Just before the trial, a mediator suggested a settlement of \$225,000, but McDonald's again refused. In part, it can be said that all the refusals by McDonald's explain the high award of damages from the court. Another often-ignored fact is that the trial judge found Mrs Liebeck to be 20 per cent contributorily negligent.²⁸

Watch this humorous video from 'Adam Ruins Everything' <<https://www.youtube.com/watch?v=Q9DXSCpcz9E>> which explains the real story of the *Liebeck* case, and how it has fed into the perception of 'compensation culture'.

‘Consumer expectation’ was also relevant in *Pollard v Tesco Stores Ltd* [2006]. There, the Court of Appeal found that the child-resistant top of a bottle of dishwasher detergent was not defective, despite the fact that in this case it had been opened by a child. The bottle top did not conform to the relevant British Design Standard, but even so, the claim was dismissed on the grounds that consumers would not generally know what the relevant design standard was; all they would expect would be that a ‘child-proof’ cap was harder for a child to remove than a normal cap. As this was still the case, the product was not defective.

Pause for reflection

Does the outcome of the *Tesco* case seem correct to you? Would you think a bottle of chemicals from which a child could remove the supposedly child-proof cap fits the definition of a ‘defective’ product under section 3 of the CPA? This certainly does not seem like strict liability. Furthermore, it seems as though the expectation of the manufacturers (who would or ought to know the relevant design standard), rather than of the public, was the driving factor in this case. The CPA, it should be remembered, specifies that the relevant expectations should be those that ‘persons generally’ (including manufacturers) are ‘entitled to expect’ when determining whether or not a product is defective (s 3). Does the outcome of this case now mean that consumers are not entitled to expect that ‘child-proof’ caps on dangerous substances are, in fact, child-proof?

p. 365 A question about whether claimants need to prove the exact way in which a defect manifests itself, or if it is enough to show that there is ‘some defect’ (a ‘broad-brush’ approach) was answered in *Hufford v Samsung Electronics (UK) Ltd* [2014]. The claimant sought to show that his fridge was defective, as it had unexpectedly and seemingly inexplicably caught fire—he could not point to the exact cause but was only operating the fridge in conditions of ‘normal use’. The defendant argued that as the claimant could not be reasonably specific about the nature of the defect, he should not succeed. HHJ Grant found:

in relation to a claim under the 1987 Act, a claimant does not have to specify or identify with accuracy or precision the defect in the product he seeks to establish, and thus prove. It is enough for a claimant to prove the existence of a defect in broad or general terms, such as ‘a defect in the electrics of the Lexus (motor car)’. (at [25])

Unfortunately, in *Hufford*, the claim eventually failed, as the claimant was unable to discharge the burden of proof that a defect in the fridge had caused the fire, rather than any other cause.²⁹

A claimant must also prove that the item they allege to be defective would be defective with ‘ordinary use’. In *Wilkes*, the claimant sought to render the defendant company, which had provided the artificial hip joint used in his hip-replacement operation, liable under the CPA. One part of the artificial hip was a steel femoral shaft called a ‘C-Stem’. Three years after Wilkes’ operation, the C-stem fractured inside his body, leaving metal debris around the joint. However, the court found that the implant failed because of the excessive and unpredictable load it had been subjected to and it was not the case that the hip implant had been subject to

normal use and had failed earlier than expected. Hickinbottom J said that no medicinal product could be absolutely safe or risk-free, and ‘persons generally’ were therefore not entitled to expect that they will be (at [65]). Again, this seems like a move further away from strict liability.³⁰

In *Gee and others v DePuy International Ltd* [2018], a case was brought by 312 individuals affected by ‘adverse reaction to metal debris’ following hip replacements, including damage to surrounding tissue and pain, swelling and numbness in the leg, resulting in the need for further corrective surgery (revision). In a lengthy judgment (170 pages), Mrs Justice Andrews DBE found that the claimants had failed to establish that the safety of the product at the relevant time (2002) did not meet the standard that the public generally were entitled to expect. The test applied was whether the product had an abnormal tendency to result in damage or harm, compared with comparable products. Factors such as cost, avoidability of the harm and the benefit to society of the product were found to be relevant.³¹

p. 366

The Council considered that the economic risk of producing a defective product should fall on the person best equipped to take precautionary steps or to insure against it, namely, the producer. That is something the Court must not forget. Nevertheless, depending on the circumstances and the product, and particularly on the harm, it may be that in an appropriate case the Court could legitimately conclude that the public would not be entitled to expect the producer to achieve something in terms of safety that is scientifically impossible or prohibitively expensive or which it would be impossible to insure against. It all depends on the nature of the product and its intended use, and all the other relevant circumstances. (at 167)

Watch our video ‘Intimate products and gendered harm’ on product liability in relation to medical devices and the gendered nature of the harms that may occur.

Video playback is not supported in this format.

Video 12: Intimate products and gendered harm

It is unlikely that this case marks the end of litigation about hip replacements (or indeed other medical devices)—further group litigations are being planned, resting on different harms and differing factual scenarios.³² In addition, 2022 marked the first time a product liability case reached the Supreme Court.

Hastings v Finsbury Orthopaedics Ltd and another (Scotland) [2022] **UKSC 19 (SC)**

In *Hastings*, another case about metal-on-metal (MoM) hip replacements, the Scottish court considered the impact of *Wilkes* and *Gee*, agreeing with the general approach but rejecting the specific aspect of *Gee* that rested on a comparison with other products, instead agreeing, as the claimant argued, that a consumer's 'entitled expectation' would be that a product would be at least as good as alternatives. The claimant later failed on causation. On appeal to the Supreme Court, his appeal was further dismissed, on the basis that:

The nature of the MITCH–Accolade product is such that there can be no entitlement to an absolute level of safety ... The test of entitled expectation ... [is] ... whether ... the level of safety of the MITCH–Accolade product would not be worse, when measured by appropriate criteria, than existing non–MoM products that would otherwise have been used. (Lord Lloyd-Jones at [19])³³

This was to be measured against two criteria: 'time to revision' and 'prospect of success of revision'. There was no evidence to support the appellant in relation to either proposition, so the claim failed. *Hastings* therefore is the latest in a long line of cases in which claimants have failed to prove a 'defect' under the CPA.

Despite there being a limited number of cases concerning the meaning of 'defect' for the purposes of applying liability under the CPA and, more to the point, that manufacturers (as can be seen in the cases discussed) seem quite easily to be able to avoid their products being classed as defective, there have been some successful claims.

p. 367

Abouzaid v Mothercare (UK) Ltd [2000] EWCA Civ 348 (CA)

The claimant, a 12-year-old boy, was injured when trying to fasten a clasp on the strap used to affix a 'cosytoes' (a detachable cover that envelops a baby's torso and legs) to a pushchair. While the elastic on the strap was stretched, he let the strap go and it recoiled, hitting him in the eye, causing injury to the retina and leaving him with impaired sight.

The court found that the product (the cosytoes) was defective. In its view, the manufacturer could have done more to prevent accidents of this type occurring, such as by using a different method to fasten the product to a pushchair, or providing a warning. Because it was so easy to avoid the danger,

and the manufacturer had not taken adequate steps to do so, the product was defective and Mothercare was liable for the harm it caused.³⁴ Put another way, the ease with which the risk of injury could have been avoided was taken into account when determining liability.

In *Abouzaid* the Court of Appeal found that the relevant test was that the product must be judged against the expectations of persons generally in all the circumstances of the case—that is, whether the product had the level of safety which the general public would expect of such a product at the time the injury occurred. As the public would not expect injury to be able to be caused by something as generally innocuous as a cosytoes, this particular product *could* be deemed unsafe, and therefore defective, because it had unexpectedly caused injury.

Pause for reflection

Compare the decision (in particular the reasoning) in *Abouzaid* with *Tesco*. Are the two decisions consistent in the way they have determined what a defective product is (or is not)? Which do you think is the better decision and why? One difference between the two cases is that a cosytoes will obviously come into close proximity with children, due to its very nature, whereas it might be expected that a cleaning chemical would be kept in a safe place away from children by responsible parents as an additional safeguard, despite it supposedly being child-proof. However, overall the decision in *Abouzaid* does seem to be more logical and ‘in tune’ with what the expectations of ‘persons generally’ might be.

A and others v National Blood Authority [2001] 3 All ER 289 (QBD)

The claimants contracted Hepatitis C, a liver disease, after being supplied with contaminated blood during blood transfusions. The people who donated the blood had been infected with the Hepatitis C virus but, at the time they donated, there was no way of testing for its presence. At the time of the transfusions the medical profession was aware that Hepatitis C existed and that it could be transmitted by a blood transfusion, but there was still no way of testing the blood. Thus, it was known that some blood provided for transfusions *could* be contaminated with Hepatitis C, but no way of telling *which* blood this was. The public was not warned of this small risk.

The court had to decide whether the contaminated blood was defective under the CPA. The defendants argued that the blood could not have been defective because the public was only ‘entitled’ to expect the product to be as safe as it could be if reasonable precautions had been taken when handling it and there was nothing more that the authority could have done to make the product safer. In short, they

argued that the risk was unavoidable and this had to be considered when the court looked at ‘all the circumstances’ that must be considered (under the CPA, s 3(2)). However, the claimants argued that to take such things into account would let ‘questions of fault back in by the back door’ (at 316).

The defendant’s argument was rejected by Burton J, who found the blood to be defective. He held that the question of avoidability should not be taken into account as section 3(2) could be interpreted to mean ‘all *relevant* circumstances’: what the producer could or could not have done, therefore, was not a *relevant* consideration when looking at strict liability (as opposed to negligence).³⁵ He defined the infected blood as a ‘non-standard’ product—one that does not perform as the producer of the product intends—adding that where there is a harmful characteristic in a non-standard product, a decision that the product is defective would be ‘straightforward’. The primary question would be whether the public accepted the non-standard nature of the product—that is, whether it could be said that the risk of some blood being affected was one that had been accepted by the public. On this point, he found that the public was entitled to expect blood that was 100 per cent safe. Therefore the blood authority was liable.

This was a very claimant-friendly decision. Burton J paid close attention to the meaning of ‘defect’ under the CPA. Because the Act itself says that it must be interpreted in line with the EC Directive, he used the definitional terms of the Directive rather than those of the Act. In so doing, he provided a three-stage test for determining whether a product is defective:

- (1) What harmful characteristic in the product caused the injury?
- (2) Was the product ‘standard’ or ‘non-standard’?
- (3) What are the consumer’s legitimate expectations as to the product?

It therefore becomes harder to establish that there is a defect in a ‘standard’ product than a ‘non-standard’ one. However, even with non-standard products, the legitimate expectations of consumers must be considered, so being non-standard clearly does not equate to being ‘defective’ in and of itself (see *Richardson v LRC Products* [2000] and now *Gee and others v DePuy International Ltd* [2018] and *Hastings v Finsbury Orthopaedics Ltd and another (Scotland)* [2022]). What consumers are entitled to expect will depend on whether any risk associated with the product is public knowledge and, crucially, whether this risk can be objectively considered socially acceptable. Infected blood, according to Burton J, clearly was not socially acceptable in any sense (at [56]).

The A judgment was the leading authority on ‘defectiveness’ for nearly 20 years. According to A, where the public would not accept the ‘non-standard’ nature of a product, the defendant would be liable. This was true irrespective of whether the ‘non-standard’ product could have been made standard (the only option being to withdraw the product from circulation) or whether the wholesale withdrawal of the product would have significant disadvantages to society. In light of *Wilkes* [2016] and *Gee* [2018], which as we saw earlier did consider other factors in arriving at the decisions that the products were non-defective, at least part of the decision must now be in doubt. The approach now to be followed appears to be that outlined in *Hastings*.

If the point of the CPA was to achieve strict liability for ‘defective’ products that cause harm, then we might consider that A was in fact correct despite it not seeming to be fair that the blood authority was held liable, as there was literally nothing it could have done at the time to have made the blood safe. Strict liability is, however, not meant to be ‘fair’, it is meant to be strict, and the desirability of the outcome (compensation for the harm suffered by the claimants) supposedly justifies this. However, A seems somewhat inconsistent with most other claims taken under the Act and it might be questioned why this is—was Burton J, perhaps, swayed by the number of potential claimants or the very serious nature of the harm done to them, or by the nature of blood transfusions being a necessary life-saving measure?

In 2017, the High Court granted a group litigation order to a group of 500 people comprising some of the surviving victims and family members of those who died following the infected blood scandal of the 1970s and 1980s. Thousands of people are estimated to have died after receiving blood products imported from the United States which were infected with Hepatitis C and HIV. As a result of an ongoing public inquiry into the scandal, announced by the then Prime Minister Theresa May in July 2017,³⁶ in August 2022 the government agreed to compensate survivors and the partners of those who died. The first interim payments of £100,000 per person were made in October 2022.³⁷ In December 2024, the first full compensation payments were accepted by victims, paid by the Infected Blood Compensation Authority (IBCA) as part of a new compensation service. Initially, ten people were offered compensation totalling over £13 million, with 25 more people invited to claim.³⁸ It may therefore matter very much which approach to defective products is followed in future.

12.3.3 Causation and limitations

If a defect is found, a claimant must still show that the defect caused the damage. The ordinary principles of legal causation apply.³⁹ Therefore, at the point of determining causation, when the claim is against the manufacturer of a medical product (e.g. a drug) in particular, the claimant would seem to be no better off than under the common law, particularly where there are multiple potential causes of the harm, especially given that the idea of ‘materially increasing the risk of harm’ in this type of case appears to have been ruled out in *Wilsher v Essex Area Health Authority* [1988].⁴⁰ This point was clearly illustrated in *XYZ v Schering Health Care Ltd* [2002], where Mackay J concluded that because the evidence suggesting a causal connection between the third-generation oral contraceptive pill and cardio-vascular problems was contradictory and insufficient, the case would be doomed to fail on causation, so there was no need to decide whether and how the CPA would apply.⁴¹

Furthermore, time limitations are placed on claims under the CPA. A claimant must bring the claim within three years of the harm being suffered (though in ‘latent harm’ situations where the claimant only discovers later that harm has been suffered, the three years run from this time, not from when the harm was literally caused). While this is quite generous, another limitation period exists, giving claimants only ten years to claim from the time the particular product was put into circulation.⁴² This is to take into account consumer expectations—arguably if a product has (successfully) been on the market for ten years, it is not defective. What these limitations mean in practice, however, is that if a product has been on the market for nine years and 11 months, and then the claimant is harmed by a defect in the product, a claim must be taken almost immediately in order not to run out of time. If injury occurs a month later, no claim will be possible.

As long as an action is initiated within the ten-year period it can be decided. In *Horne-Roberts v SmithKline Beecham* [2001], the Court of Appeal ruled that this was the case even where proceedings had originally been mistakenly initiated against the *wrong* producer. The correct producer was later allowed to be substituted in the action even though the ten-year period had by then expired. In 2008, the House of Lords referred this issue to the European Court of Justice (ECJ), asking for clear guidance on the position where a claimant mistakenly names the wrong producer in an action commenced within the limitation period.⁴³ The ECJ ruled that certain circumstances would permit substitution of a producer after the expiry of the time limit, despite the fact this limit was strict. These included those where the party sued is a wholly owned subsidiary of the producer, and 'the putting into circulation of the product' had in fact been determined by that producer.⁴⁴ Upon the case's return (then to the Supreme Court), it was unanimously found that domestic law did not allow a producer to be substituted as the defendant in place of a wholly owned subsidiary, unless the parent company had actually determined when the supplier put the product in circulation. On the facts, this had not happened and the appeal was unsuccessful—the claimant was out of time.⁴⁵

12.3.4 Defences

The availability of defences in the CPA appears to allow manufacturers to say 'it was not my fault'. Strict liability, by definition, ought not to be fault-based.⁴⁶ While the presence of explicit defences in the CPA is controversial, it may be argued that liability under the Act should be strict, but not absolute.

p. 371 ↪ However, of the six defences built into the CPA in section 4,⁴⁷ one in particular has proved especially contentious. Under section 4(1)(e) a manufacturer or other party who finds themselves liable under the Act can claim:

that the state of scientific and technical knowledge at the relevant time was not such that a producer of products of the same description as the product in question might be expected to have discovered the defect if it had existed in his products while they were under his control.

This has become known as the 'development risks' or 'state of the art' defence. It covers circumstances where, sometime after the product was produced, scientific or technical knowledge renders it 'defective'. Put another way, if a discovery is made after the product is put into circulation by the manufacturer, meaning that the product would now be regarded as defective, the manufacturer escapes liability (on that occasion only, as any time after the first time the defect would be known). Ironically, therefore, under the CPA the Thalidomide claimants are unlikely to have been better off, despite this tragedy and other pharmaceutical harms being key factors leading to the movement to strict liability.⁴⁸ As Colin Scott, Julia Black and Ross Cranston have pointed out, large pharmaceutical companies operating from the UK are the biggest beneficiaries of the inclusion of the (optional) defence.⁴⁹ In fact, the UK Government was one of the most vocal when it came to implementing the Directive and was very much in favour of not using the derogation provision that would have allowed the defence to be excluded.

Pause for reflection

Does the development risks defence sound, in essence, like a crude version of the *Bolam* test in negligence which applies to professionals when determining whether they have fallen below the expected standard of care? In that test, if the professional concerned can show that they acted in the same way as others from a respectable body of professional opinion would have done, they will not be in breach of their duty of care. This makes it harder to find professionals liable in negligence.⁵⁰ The development risks defence, as it is phrased in section 4(1)(e), appears to give manufacturers the ability to say that ‘other manufacturers would not have been able to spot the defect, given the state of scientific and technical knowledge’. Do you think this is what it means? Is this a subjective or an objective test?

Case C-300/95 *Commission of the European Communities v United Kingdom*, EU:C:1997:255 (ECJ) (also known as *EC v UK*)

The CPA was meant to implement the terms of the EC Directive. The European Commission was concerned that the terminology of section 4(1)(e) of the CPA (the ‘development risks defence’) deviated from the wording of the defence under Article 7 of the Directive, creating what could be called a subjective test, as it focused on the conduct and abilities of the ‘reasonable manufacturer’. Article 7(e) had been differently worded and required an objective assessment of the state of scientific and technical knowledge at the time the product was put into circulation. It said that the defence would apply when:

[t]he state of scientific and technical knowledge at the time when the producer put the product into circulation was not such as to enable the existence of the defect to be discovered.

In *EC v UK*, the ECJ said that the relevant test was to ask whether the information (that would make the product defective) was ‘accessible’ to the producer of the product concerned at the relevant time.

Relying on ‘accessibility’ of knowledge means that even if someone in the world might know the product is unsafe (because this is shown by later scientific or technical discoveries), the *producer* might not necessarily be able to know this. This, however, appears to be a subjective rather than objective assessment. The ECJ suggested that this meant, for example, that if a scientific discovery was made in a remote part of Manchuria (a region of China), and written up only in an obscure Manchurian journal (or not at all), then this information will not be ‘accessible’ to all producers.⁵¹ However, if a discovery is made in France, even if this was written up in French and published in a French journal, the information would be likely to be deemed to

be accessible to the majority of manufacturers (particularly European manufacturers covered by the Directive). In short, accessible information means that risks become foreseeable, and therefore the defence cannot apply.

In *A*, the defendant blood authority also contested the claim on the basis that even if a defect in the product could be established, they could avail themselves of the development risks defence. Burton J, however, ruled that the defence was not applicable, as the general risk that the blood was contaminated with the virus was known (albeit unavoidable, as this knowledge did not extend to being able to find out whether a particular bag of blood was defective). This decision means that the defence could only be used the very first time a risk becomes known. The blood authority knew the risk existed, so the defence could no longer apply.

Pause for reflection

Do you think the National Blood Authority should have been found liable, especially given that it is obliged under statute to provide blood and could not at the time screen for the presence of the Hepatitis C virus in blood samples?

Check our **pointers** <<https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-12-pointers-to-pause-for-reflection-and-counterpoint-boxes?options=showName>> on how to answer the questions posed in this 'pause for reflection'.

12.3.5 Overall effect of the Consumer Protection Act 1987

p. 373 There have been very few cases under the CPA. While this might be in part due to high litigation costs, it might be argued that the Act has had a more silent effect in that the notion that manufacturers can be found to be strictly liable for harm caused by their products, even without negligence, may encourage more out-of-court settlements or manufacturers to make ↵ their products safer. It might have been thought that the decision in *A*, where strict liability appeared to be taken to its strictest point, would mean that more claims would be brought by consumers. However, this does not appear to have happened and more recent cases tend to suggest that there is little benefit to bringing a claim under the Act rather than a claim in negligence, as well as a move away from the consumer-friendliness of *A*. Considerations of 'fault' seem to have crept into the CPA, particularly in the sense of what a defect can be said to be, despite Burton J's attempts in *A* not to 'dilute' the intentions underlying the CPA.

Counterpoint

It also appears that the development risks defence undermines the aims of the CPA. Arguably, what it does is switch the burden of proof onto producers to show that when they designed the product that has caused harm, they took the care expected of an expert/manufacturer at that time. In this respect, although strict liability in general and the CPA in particular were meant to make things easier for claimants in terms of *design* defects (remember that at common law the duty is clear and the courts are often willing to infer that a manufacturer was in breach (*Grant*),⁵² meaning that it is relatively easy to sue for defects caused by the *manufacturing* process), it does not seem to have improved much at all. Claimants in the Thalidomide cases, for example, would have been no better off than they were in taking their claims in negligence. This, combined with the small number of cases brought under the CPA (and the even smaller number of successful claims), seems to suggest that it has been largely ineffective in achieving its supposed aims.⁵³

Manufacturers have argued throughout the history of the implementation of the CPA that strict liability would, for example, hinder innovation and the placement of new, better and more cost-effective products onto the market. Strict liability, in their eyes, leads to increased costs, which in turn must be passed on to consumers. In turn, this leads to the question of whether private law mechanisms are an appropriate response to defects in products at all, or whether increased public law regulation would better benefit society. In relation to strict liability for products, the argument is whether we feel it is better that manufacturers/producers should bear the costs of unknown risks (through insurance?) or whether consumers, through product choice, would be better equipped to make decisions based on the level of risk they are prepared to accept. Forcing producers to compensate through a strict liability regime forces product prices up (as producers bear all, or most, of the risk). In any case, imposing new or additional burdens on producers in the current economic climate would seem very unlikely.

However, the inhibition of innovation and design does not seem to have occurred in the nearly 40 years since the CPA came into force—perhaps again suggesting that the Act itself is not as ‘strict’ as it could have been in imposing liability on manufacturers and that the behaviour of producers is unlikely to have been significantly altered by the presence of the Act.

Test your understanding by trying this section’s self-test questions <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-12-3>.

p. 378 **12.4 Part 1 of the Consumer Protection Act 1987—Annotated**

In **Figure 12.1** we have annotated Part 1 of the Consumer Protection Act 1987, highlighting the key issues discussed in this chapter.

Access an interactive annotated copy of the Act <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-12-annotated-statute-the-consumer-protection-act-1987-figure-12-2-interactive> and click on the highlighted phrases for author commentary. You can also download a non-interactive copy of the annotated extract <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-12-annotated-statute-the-consumer-protection-act-1987-figure-12-1>.

The Consumer Protection Act 1987

Part 1

Anything in square brackets means this is a section inserted by later legislation or regulations.

'Producers' are those that can be held liable under the Act – as the rest of this section reveals (see emboldened text), the concept of who a producer is has been quite broadly defined.

1.—(1) This Part [was enacted] for the purpose of making such provision as [was] necessary in order to comply with the product liability Directive and shall be construed accordingly.

(2) In this Part, except in so far as the context otherwise requires—

“dependant” and “relative” have the same meaning as they have in, respectively, the Fatal Accidents Act 1976 and the [Damages (Scotland) Act 2011];

“producer”, in relation to a product, means—

(a) the person who manufactured it;

(b) in the case of a substance which has not been manufactured but has been won or abstracted, the person who won or abstracted it;

(c) in the case of a product which has not been manufactured, won or abstracted but essential characteristics of which are attributable to an industrial or other process having been carried out (for example, in relation to agricultural produce), the person who carried out that process;

“Product” means any goods or electricity and (subject to subsection (3) below) includes a product which is comprised in another product, whether by virtue of being a component part or raw material or otherwise;

and

“the product liability Directive” means the Directive of the Council of the [European Union], dated 25th July 1985, (No. 85/374/EEC) on the approximation of the laws, regulations and administrative provisions of the member States concerning liability for defective products.

(3) For the purposes of this Part a person who supplies any product in which products are comprised, whether by virtue of being component parts or raw materials or otherwise, shall not be treated by reason only of his supply of that product as supplying any of the products so comprised.

2.—(1) Subject to the following provisions of this Part, where any damage is caused wholly or partly by a defect in a product, every person to whom subsection (2) below applies shall be liable for the damage.

(2) This subsection applies to—

(a) the producer of the product;

It was Directive 85/374/EEC that the UK had to implement, and did so by passing Part 1 of the CPA 1987 – see the definition in s 1(2).

See Chapter 21.

Definition of ‘products’ for the purposes of the Act – see also s 45. The case **A v National Blood Authority [2001]** says that blood received in transfusions is also a ‘product’ covered by the Act.

This is what makes a defendant liable – if they produce a product that has a defect which causes harm.

See definition of ‘producer’ in s 1.

→

This means that 'own-branders' can be construed as producers, even if they did not actually make the product but someone else made it for them – e.g. Co-op's own-brand cornflakes. However, the situation is less clear if the distinction 'made for X' or 'selected for X' is used on a product – does this imply that the own-brander does not 'hold himself out to be the producer'?

For an explanation of this term, see the chapter on damages, **Chapter 21**.

People are generally entitled to expect what is 'socially accepted' as a risk that comes with the product – see e.g. *Richardson v LRC Products* [2000]; *Bogle v McDonald's* [2002] and contrast *A v National Blood Authority* and *Hastings*.

e.g. the packaging.

e.g. industry standard marks such as the 'Kite Mark'.

(b) any person who, by putting his name on the product or using a trade mark or other distinguishing mark in relation to the product, has held himself out to be the producer of the product;

(c) any person who has imported the product into [the United Kingdom] in order, in the course of any business of his, to supply it to another.

(3) Subject as aforesaid, where any damage is caused wholly or partly by a defect in a product, any person who supplied the product (whether to the person who suffered the damage, to the producer of any product in which the product in question is comprised or to any other person) shall be liable for the damage if—

(a) the person who suffered the damage requests the supplier to identify one or more of the persons (whether still in existence or not) to whom subsection (2) above applies in relation to the product;

(b) that request is made within a reasonable period after the damage occurs and at a time when it is not reasonably practicable for the person making the request to identify all those persons; and

(c) the supplier fails, within a reasonable period after receiving the request, either to comply with the request or to identify the person who supplied the product to him.

...

(5) Where two or more persons are liable by virtue of this Part for the same damage, their liability shall be joint and several.

(6) This section shall be without prejudice to any liability arising otherwise than by virtue of this Part.

3.—(1) Subject to the following provisions of this section, there is a defect in a product for the purposes of this Part if the safety of the product is not such as persons generally are entitled to expect; and for those purposes 'safety', in relation to a product, shall include safety with respect to products comprised in that product and safety in the context of risks of damage to property, as well as in the context of risks of death or personal injury.

(2) In determining for the purposes of subsection (1) above what persons generally are entitled to expect in relation to a product all the circumstances shall be taken into account, including—

(a) the manner in which, and purposes for which, the product has been marketed, its get-up, the use of any mark in relation to the product and any instructions for, or warnings with respect to, doing or refraining from doing anything with or in relation to the product;

(b) what might reasonably be expected to be done with or in relation to the product; and

(c) the time when the product was supplied by its producer to another;

and nothing in this section shall require a defect to be inferred from the fact alone that the safety of a product which is supplied after that time is greater than the safety of the product in question.

This section provides that even suppliers (retailers) can be liable in certain (limited) circumstances, e.g. where the retailer either cannot or does not identify the producer.

Definition of 'defect' – the key part of the Act and what most of the case law pertains to.

Controversially redefined by Burton J in *A v National Blood Authority* as 'all the relevant circumstances', excluding consideration of whether reasonable care was taken by the manufacturer. Now see *Hastings*.

Instructions or warnings with a product can render it 'safe' and therefore not defective – see e.g. *Worsley v Tambrands* [2000].

→

4.—(1) In any civil proceedings by virtue of this Part against any person ('the person proceeded against') in respect of a defect in a product it shall be a defence for him to show—

It is controversial that any defences are built into the Act—but the liability is 'strict', not 'absolute'.

(a) that the defect is attributable to compliance with any requirement imposed by or under any enactment or with any [assimilated] obligation; or

(b) that the person proceeded against did not at any time supply the product to another; or

(c) that the following conditions are satisfied, that is to say—

(i) that the only supply of the product to another by the person proceeded against was otherwise than in the course of a business of that person's; and

(ii) that section 2(2) above does not apply to that person or applies to him by virtue only of things done otherwise than with a view to profit; or

(d) that the defect did not exist in the product at the relevant time; or

(e) that the state of scientific and technical knowledge at the relevant time was not such that a producer of products of the same description as the product in question might be expected to have discovered the defect if it had existed in his products while they were under his control; or

The 'development risks' or 'state of the art' defence. This is the most controversial of all the defences and the wording used in the CPA was challenged by the European Commission in **EC v UK [1997]**. They alleged that the CPA definition allowed a subjective interpretation of what a 'reasonable producer' could have been expected to know at the time, when the standard set by the Directive was meant to be objective. In that case it was deemed that all 'accessible' information or knowledge had to be considered.

(f) that the defect—

(i) constituted a defect in a product ("the subsequent product") in which the product in question had been comprised; and

(ii) was wholly attributable to the design of the subsequent product or to compliance by the producer of the product in question with instructions given by the producer of the subsequent product.

...

5.—(1) Subject to the following provisions of this section, in this Part 'damage' means death or personal injury or any loss of or damage to any property (including land).

Definition of 'damage'—i.e. what harms can be claimed for under the CPA. It is surprising, perhaps, that this definition comes so late.

This would be pure economic loss in negligence and is also irretrievable under the Act.

(2) A person shall not be liable under section 2 above in respect of any defect in a product for the loss of or any damage to the product itself or for the loss of or any damage to the whole or any part of any product which has been supplied with the product in question comprised in it.

(3) A person shall not be liable under section 2 above for any loss of or damage to any property which, at the time it is lost or damaged, is not—

(a) of a description of property ordinarily intended for private use, occupation or consumption; and

→

→

(b) intended by the person suffering the loss or damage mainly for his own private use, occupation or consumption.

(4) No damages shall be awarded to any person by virtue of this Part in respect of any loss of or damage to any property if the amount which would fall to be so awarded to that person, apart from this subsection and any liability for interest, does not exceed £275.

(5) In determining for the purposes of this Part who has suffered any loss of or damage to property and when any such loss or damage occurred, the loss or damage shall be regarded as having occurred at the earliest time at which a person with an interest in the property had knowledge of the material facts about the loss or damage.

(6) For the purposes of subsection (5) above the material facts about any loss of or damage to any property are such facts about the loss or damage as would lead a reasonable person with an interest in the property to consider the loss or damage sufficiently serious to justify his instituting proceedings for damages against a defendant who did not dispute liability and was able to satisfy a judgment.

(7) For the purposes of subsection (5) above a person's knowledge includes knowledge which he might reasonably have been expected to acquire—

(a) from facts observable or ascertainable by him; or

(b) from facts ascertainable by him with the help of appropriate expert advice which it is reasonable for him to seek;

but a person shall not be taken by virtue of this subsection to have knowledge of a fact ascertainable by him only with the help of expert advice unless he has failed to take all reasonable steps to obtain (and, where appropriate, to act on) that advice.

(8) Subsections (5) to (7) above shall not extend to Scotland.

...

To take a claim for property damage, the loss must amount to more than £275. This means small claims are excluded under the Act.

Figure 12.1 The Consumer Protection Act 1987, Part 1

p. 378 12.5 Claiming under Part 1 of the Consumer Protection Act 1987—At a glance

See Table 12.1.

Table 12.1 Summary of the Consumer Protection Act 1987, Part 1. You can also download a copy of this table <<https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-additional-material-chapter-12-cpa-at-a-glance-table?options=showName>>.

Issue	Relevant sections	Explanation
Who can sue?	Sections 2(1) and 5(1)	A person who 'suffers damage as a result of a defective product' (see definitions of 'damage' and 'defect')
Who can be sued?	Sections 1(2) and 5(2)	Manufacturers, producers, 'own-branders' and importers into the United Kingdom. Some suppliers can also be sued
What damage can be claimed for?	Section 5(1) See also ss 5(2) and 5(4)	Death, personal injury and property damage Pure economic loss excluded and property damage must exceed £275
What is a 'product'?	Section 1(2) See also s 45	Goods and electricity, component parts and raw materials 'Goods' includes substances, growing crops, things on land and any ship, aircraft or vehicle
What is a 'defect'?	Section 3(1) See also s 3(2) (a)–(c)	A defect exists when 'the safety of a product is not such that persons generally are entitled to expect'—this is the most analysed aspect of the legislation in case law Packaging, normal use, instructions and warnings may be taken into account
Are any defences appropriate?	Section 4(1)(a)–(f)	There are six defences available under the Act Contributory negligence is also applicable

12.6 Conclusion

In this chapter we have considered the ways in which liability for defective products is regulated in tort law. This should be understood against a background knowledge of the provisions of *contract law*, which provide the backdrop to the development of the law of negligence in this area. However, despite this protection, as well as reform to the doctrine of privity of contract, many end-users of products remain unable to take contractual claims in relation to faulty or defective products. There are two main reasons for this. First, end-users who did not purchase the product (and thereby enter a contract with the retailer) usually cannot sue despite changes to the doctrine of privity. Secondly, under contract it is impossible to directly ↵ sue the manufacturer, which in many cases will be the preferred option. Thus the law of contract has limitations in terms of consumer protection.

This chapter outlined the way the tort of *negligence* has been used by claimants who suffer damage because of a defective product. As in all negligence claims, the hurdles of establishing duty, breach and causation must be overcome. While the first two are not generally difficult (as *Donoghue* gives the duty and a breach is often

inferred on the facts), causation issues can often stand in the way of a successful claim. This is particularly true when considering design defects and it was recognition of these limitations in negligence that led in part to calls for strict liability in this area.

Strict liability (where the producer of a product is held liable without the need for the injured party to show fault) for defective products was meant to have been introduced by the enactment of Part 1 of the CPA. However, the Act's definition of defect is weak, being based on what 'persons generally are entitled to expect', and much of the little case law has been devoted to finding that products are *not* defective. Furthermore, the inclusion of defences in the Act, particularly the '*development risks defence*', seems to suggest that liability remains somewhat subjective and contains elements of fault, thus not rendering it totally strict.

End-of-chapter questions

After reading the chapter carefully, try answering the questions which follow.

- 1 What are the advantages and disadvantages of having a system of strict liability in relation to defective products? Does strict liability make more sense when the harm suffered is personal injury?
- 2 Who should bear the risks and costs associated with innovation and increased consumer choice?
- 3 Do the provisions of Part 1 of the Consumer Protection Act 1987 achieve their aims?

If you would like to know what we think, check the **answer guidance** <<https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-12-answers-to-end-of-chapter-questions?options=showName>>.

Answering the problem question

Consider again the problem question at the start of this chapter. Now having read about the topic, **what would be your advice to the claimants?**

Here are some pointers to get you started:

→ The claimants would need to sue the 'producer', RHP, under s 1 of Part 1 of the Consumer Protection Act 1987, if they have suffered harms that are recognised under the Act (s 5). They would also need to establish that the drug is a 'product' covered by the Act. If they succeed in their claim, RHP will be strictly liable (i.e. without the need to prove fault), subject to any defences.

→ The central question is whether the drug was 'defective' (s 3). The different cases looking at the meaning of 'defect' should be considered, especially the most recent (and only Supreme Court) decision in ***Hastings***.

→ If it is, does RHP have any defences? Of particular interest here would be the ‘development risks defence’ (s 4(1)(e)). One criterion is that any knowledge that was published should have been ‘accessible’ (**EC v UK**)—was it?

If you need some more guidance:

→ See the **annotated version of the problem with issues and cases to consider** <<https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-12-annotated-problem-question-interactive>>.

→ Check your answer against the **suggested outline answer** <<https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-12-answers-to-problem-question>>.

Further reading

Further reading on this subject often centres on critiques of the Consumer Protection Act 1987 and in particular the development risks defence. More recent work includes Eisler and Nolan, who both provide critical commentary on the decision in *Wilkes* about how ‘defects’ should be determined (though note both are pre-*Gee*), while reaching somewhat different conclusions.

Department of Trade and Industry *Guide to the Consumer Protection Act 1987*

Eisler, Jacob ‘One Step Forward and Two Steps Back in Product Liability: The Search for Clarity in the Identification of Defects’ [2017] *Cambridge Law Journal* 233

Fairgrieve, Duncan and Geraint Howells ‘General Product Safety—A Revolution Through Reform?’ (2006) 69 *Modern Law Review* 59

Gerling, Andrea ‘A Matter of Degree: How a Jury Decided That a Coffee Spill is Worth \$2.9 Million’ *Wall Street Journal* 1 September 1994

Giliker, Paula ‘Strict Liability for Defective Goods: The Ongoing Debate’ (2003) 24 *Business Law Review* 87

Howells, Geraint and Mark Mildred ‘Infected Blood: Defect and Discoverability: A First Exposition of the EC Product Liability Directive’ (2002) 65 *Modern Law Review* 95

Newdick, Christopher ‘The Development Risks Defence of the Consumer Protection Act 1987’ [1988] *Cambridge Law Journal* 455

Nolan, Donal ‘Strict Product Liability for Design Defects’ (2018) 134 *Law Quarterly Review* 176

Wuyts, Daily ‘The Product Liability Directive—More Than Two Decades of Defective Products in Europe’ (2014) 5 *Journal of European Tort Law* 1

Additional suggestions for further reading can be found in this chapter's **web links** <<https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-12-web-links?options=showName>>.

Notes

- ¹ The facts of *Donoghue* are set out in **section 2.2**.
- ² See **Chapter 7**.
- ³ See e.g. Scrutton LJ in *Hodge v Anglo-American Oil Co* [1922], who described goods rendered 'dangerous' by negligent construction as 'a wolf in sheep's clothing' (at 187).
- ⁴ See *Wormleighton v Cape Intermediate Holdings Ltd* [2024] EWHC 1971 (KB).
- ⁵ Discussed in **section 7.5.3**.
- ⁶ See **Chapter 7**.
- ⁷ This theory is also discussed in **Chapter 7** in relation to *Murphy* (see Counterpoint box in **section 7.4**).
- ⁸ Although this has now been ruled out for buildings in the sense that foundations cannot be regarded as a separate component of a building.
- ⁹ The idea was considered in *Finesse Group Ltd v Bryson Products* [2013], see in particular [28]–[29].
- ¹⁰ See also the (*obiter*) comments of Lloyd LJ in *Aswan Engineering v Lupdine Ltd* [1987].
- ¹¹ For explanation of these different types of damages, see **section 21.2**.
- ¹² Examples of producers recalling products with dangerous defects include Samsung recalling its Galaxy phone (see Cristina Criddle 'Why is the Samsung Galaxy Note 7 catching fire?' *The Telegraph* 11 October 2016) and Whirlpool's tumble dryers (BBC News 'Whirlpool told to recall dryers in "unprecedented" government move' 12 June 2019).
- ¹³ See **Chapter 9**.
- ¹⁴ *Howmet Ltd v Economy Devices Ltd and others* [2016].
- ¹⁵ So Tyler, in the earlier scenarios, may have difficulties establishing that the drug was the cause of his stomach ulcer.
- ¹⁶ This does not mean that similar scandals cannot happen. More recently, campaigners highlighted the use of sodium valproate (used to treat epilepsy, bipolar disorder and other conditions) by pregnant women, despite being known by the medical profession to be teratogenic. See Sarah Boseley 'Birth defect risks of sodium valproate "known 40 years ago"' *The Guardian* 26 September 2017.
- ¹⁷ In fact, scientists only discovered *how* the drug caused the side effects it did in 2009, though this still would not prove that the drug *did* do this in any individual case, as other (genetic) factors could also cause the same symptoms. For a discussion of the difficulties posed by multiple potential causes, see **section 9.2.2.1**. Thalidomide manufacturers have never admitted negligence, though they have issued apologies—see BBC News 'Thalidomide apology insulting, campaigners say' 1 September 2012.
- ¹⁸ Directive 85/374/EEC.

¹⁹ This is kept under almost constant review. In 2017, the European Commission evaluated the performance of the Directive, including whether it continues to meet consumers' and others' needs—and, interestingly, whether it is fit for purpose in an increasingly technological age, with specific reference to the 'Internet of Things' (European Commission 'Public Consultation on the rules on liability of the producer for damage caused by a defective product' 10 January 2017)—it received responses from 657 stakeholders. The fifth report on the Product Liability Directive was published in 2018, concluding that despite products having increased in complexity since the Directive was issued, and although some legal terms (including 'defect' and 'product') may need better definition (especially in light of emerging technologies), it remained an adequate tool. The Commission established an expert group on liability and new technologies to keep these areas under review and to 'issue guidance on the Product Liability Directive and a report on the broader implications for, potential gaps in and orientations for, the liability and safety frameworks for artificial intelligence, the Internet of Things and robotics'. Its report was published in February 2020. Latterly, EU member states voted on a new Product Liability Directive (2022/0302) to replace the 1985 Directive. The new Directive was published on 18 November 2024 and its provisions will come into force in all member states by 9 December 2026.

²⁰ Brought into force 1 March 1988. As the UK is no longer part of the EU, the new Directive will not come into force here, meaning that there will be significant differences in how product liability claims will be handled in the UK compared to in EU member states. It will be for the government to decide how far to align with the new EU Directive.

²¹ You may find it helpful to read this section alongside the annotated version of the CPA (**section 12.4**) or the CPA 'at a glance' table in **section 12.5**.

²² See e.g. *Busby v Berkshire Bed Co Ltd* [2018] at [21].

²³ e.g. blood products provided for transfusion purposes: ***A v National Blood Authority* [2001]**. The original definition did not include agricultural produce or game, unless this had been subjected to an industrial process (e.g. in a meat-packing factory). EU member states were free to include or exclude this produce under the 1985 Directive. However, though a later Directive made inclusion compulsory after the BSE ('mad cow disease') crisis, the requirement has since been removed again. Agricultural produce is therefore not included within the definition of 'product' under the CPA.

²⁴ See also *Busby v Berkshire Bed Co Ltd* [2018], where the claimant fell off her new bed during sex, resulting in paralysis. Despite a finding of fact that the bed was not 'of satisfactory quality' (for contract law purposes), it was not found to be defective under the CPA, and her injury had been merely an accident which 'required a most unfortunate and unusual combination of positioning on the bed and movement which I do not believe would have been foreseeable by any reasonable person prior to the incident' (HHJ Cotter KC at [142]).

²⁵ New rules on cigarette packaging came into force in May 2016 after tobacco firms lost a legal challenge (*British American Tobacco UK Ltd and others, R (on the application of) v Secretary of State for Health* [2016]). Packets of ten cigarettes are no longer available and for larger packs standardised green packaging is used. Pictures depicting the harmful effects of smoking must now cover 65 per cent of the front and back of every packet of cigarettes, with additional warnings on the top of the pack.

²⁶ Controversially, since *Rees v Darlington Area Health Authority* [2002] a 'conventional' award of £15,000 for loss of autonomy may now be awarded in such cases, though this in no way compensates for the financial losses caused by the negligence.

²⁷ It should be noted here that one of the major differences—and possibly a contributing factor to the commonly held perception that in the US you can sue for anything—between the US and UK tort systems is the presence of a jury on tort cases. Typically, jurors tend to feel more sympathy to claimants (and antipathy to defendants) and this may be

particularly true where cases are taken against large corporations. Jurors also ‘decide’ damages awards, and in the US there is far more scope for non-compensatory damages (e.g. punitive or aggravated damages) than in the UK (see **Chapter 21**). In a 1998 case against Philip Morris tobacco (one of the world’s largest producers of cigarettes), a jury awarded \$81 million to the widow and children of a man who had smoked the company’s Marlboro cigarettes for 40 years (since before the time tobacco companies acknowledged the dangers of smoking). The family had sued for \$100 million but the award was reduced as the court found the man 50 per cent responsible for the harm he suffered. Less than two months later, in another claim against Philip Morris, another jury awarded \$51.5 million to a cigarette smoker with terminal lung cancer (though this was later appealed). More recently, in 2018, Monsanto (now owned by Bayer) was found liable for gardener DeWayne Johnson’s terminal cancer caused by ‘Roundup’, one of the company’s weedkiller products. The jury awarded him \$289 million damages. There have also been subsequent successful cases, with substantial damages awards. See also Patricia Cohen ‘Roundup maker to pay \$10 billion to settle cancer suits’ *The New York Times* 24 June 2020—reportedly one of the biggest settlements in US litigation history.

²⁸ See **Chapter 10**.

²⁹ See also *Ide v ATB Sales Ltd and another* [2008].

³⁰ Contrast the decision in an Australian case brought on behalf of hundreds of women left with excruciating pain and other symptoms due to transvaginal mesh implants. In *Gill v Ethicon Sàrl (No 5)* [2019], Katzmann J found that lack of an adequate warning affected consumer expectations and that ‘[w]hat persons generally are entitled to expect of medical devices is also affected by what the manufacturer says or does not say about them’ (at 3370). The producers, Johnson & Johnson, were subsequently unsuccessful in an appeal.

³¹ Note, however, that the product in question was removed from sale in 2013.

³² Note also that new Regulations are due to be enacted following the Independent Medicines and Medical Devices Safety Review of 2020 (‘Cumberledge Review’), which was highly critical of the failure of the UK’s existing regulations to protect some categories of patient from the side effects of medical devices. There is concern that some products, such as surgical mesh, require more regulatory oversight and that the UK has fallen behind compared to international practice (notably, by comparison with EU regulation, which was updated in 2017 but due to quirks of Brexit, not adopted by the UK). The Medicines and Healthcare products Regulatory Agency (MHRA) conducted a consultation in 2021 and the government issued its response in June 2022—this is not necessarily the final word but does indicate what may likely follow, including that manufacturers must have insurance in place to meet the costs of compensation to patients and users of medical devices who are injured by them.

³³ *Hastings* [2022].

³⁴ This may partly have been because of the manufacturer’s own reputation as a leader in the baby products market—see Pill LJ at [27].

³⁵ It should be noted that this determination may not have survived an appeal had this case gone further. It is also potentially in doubt after *Gee* and *Hastings*.

³⁶ See Jim Duffy ‘Contaminated blood: statutory inquiry announced’ *UK Human Rights blog* 7 November 2017. The progress of the Inquiry can be found at www.infectedbloodinquiry.org.uk/news <<http://www.infectedbloodinquiry.org.uk/news>>.

³⁷ In November 2022, it was revealed one in three of those infected were children: BBC News ‘1 in 3 infected with HIV in blood scandal was a child’ 9 November 2022.

³⁸ In its first budget after coming to power in 2024, the Labour government set aside £11.8 *billion* for the compensation scheme (see Cabinet Office Press Release ‘First compensation offered to infected blood victims after decades of injustice’ 12 December 2024: www.gov.uk/government/news/first-compensation-offered-to-infected-blood-victims-after-decades-of-injustice <<https://www.gov.uk/government/news/first-compensation-offered-to-infected-blood-victims-after-decades-of-injustice>>).

³⁹ As outlined in **Chapter 9**.

⁴⁰ For a full discussion of the development and treatment of the concept of ‘material increase in risk’, see **section 9.2.2.1**.

⁴¹ The reason for the failure of the claim in *Busby v Berkshire Bed Co Ltd* [2018] was that the injury had not been caused by the bed, but by the activity of the claimant (at [105]). Though this was the case, HHJ Cotter KC also considered whether there was a defect under the CPA and/or a breach of contract and/or negligence.

⁴² Limitation Act 1980, s 11A(3).

⁴³ *O’Byrne v Aventis Pasteur SA* [2008]. The ECJ had already once provided guidance on the matter, in *O’Byrne v Sanofi Pasteur MSD Ltd* [2006], though evidently this was not clear.

⁴⁴ *Aventis Pasteur v O’Byrne* [2009].

⁴⁵ *O’Byrne v Aventis Pasteur SA* [2010].

⁴⁶ Compare the Vaccine Damage Payments Act 1979, under which a tax-free statutory sum is awarded to people who become severely disabled as a result of vaccination against certain diseases, without proof of fault. The one-off lump sum payment increased from £100,000 to £120,000 in July 2007, having previously increased from £40,000 to £100,000 in 2000. In December 2020, the Department of Health and Social Care announced that the government was adding Covid-19 vaccines to the Vaccine Damage Payments Scheme.

⁴⁷ Contributory negligence is also available though not listed in s 4.

⁴⁸ Which is exactly why the Law Commission recommended that parties injured by defective products should be compensated no matter what care had been taken by the producer (*Liability for Defective Products* (Law Com No 82, Cmnd 6831, 1977)).

⁴⁹ Colin Scott, Julia Black and Ross Cranston *Cranston’s Consumers and the Law* (CUP 2000) 194.

⁵⁰ See **section 8.3.2**.

⁵¹ **EC v UK [1997]**, Opinion of AG Tesouro at [23].

⁵² It seems that this is also a possibility under the CPA: see *Ide v ATB Sales* [2008].

⁵³ Though of course it may have a ‘hidden’ effect in encouraging either out-of-court settlements or swift(er) recall of defective products by manufacturers. See e.g. David Sanderson ‘Hundreds burnt by toxic sofas to share £20 million compensation’ *The Times* 27 April 2010; BBC News ‘Pushchair maker Maclaren agrees compensation’ 6 May 2010; BBC News ‘Robinsons Fruit Shoot children’s drink in safety recall’ 3 July 2012; BBC News ‘Aston Martin recalls 17,000 cars over possible defective part’ 5 February 2014; BBC News ‘Aldi chocolate recalled in salmonella scare’ 5 January 2015; Hilary Osborne ‘Ikea recalls child safety gates following accidents’ *The Guardian* 23 June 2016; Mared Gruffydd ‘B&Q adds another heater to recall list amid electric shock and fire fears’ *Daily Express* 20 March 2021.

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